

Cost-Effectiveness of Intismeran Autogene Plus Pembrolizumab Versus Pembrolizumab Alone as Adjuvant Therapy for Resected Stage IIIB–IV Melanoma: An Updated Early Economic Evaluation

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Abstract

Background: On August 19, 2026, Merck and Moderna announced positive topline results from the Phase 3 INTerpath-001 trial, establishing intismeran autogene (mRNA-4157/V940) plus pembrolizumab as the first mRNA-based individualized neoantigen therapy (INT) to succeed in a late-stage trial. Intismeran plus pembrolizumab significantly improved recurrence-free survival (RFS) and distant metastasis-free survival (DMFS) versus pembrolizumab alone in patients with completely resected stage IIIB–IV melanoma. However, the economic value of this novel combination remains unknown.

Objective: To estimate the cost-effectiveness of intismeran plus pembrolizumab versus pembrolizumab alone as adjuvant therapy for resected stage IIIB–IV melanoma from a US payer perspective.

Methods: A four-state partitioned survival model (recurrence-free → locoregional recurrence → distant metastasis → death) was developed with a lifetime horizon (40 years) and 3% annual discounting. Clinical efficacy data were derived from the 5-year follow-up of the Phase 2b KEYNOTE-942 trial. Parametric survival functions (log-normal) were calibrated to RFS, DMFS, and OS survival probabilities at 18, 24, 36, 48, and 60 months, with the ratio-constrained structure $RFS \leq DMFS \leq OS$ ensuring internal consistency. Health state utilities were derived from published melanoma EQ-5D literature (RF 0.83, LR 0.64, DM 0.55). Sensitivity analyses included probabilistic sensitivity analysis (1,000 iterations), deterministic sensitivity analysis, and scenario analyses.

Results: In the base case, with general population mortality constraint applied from 60 months, intismeran plus pembrolizumab yielded 11.10 QALYs at a cost of \$860,118, compared with 7.34 QALYs at \$713,157 for pembrolizumab alone. The incremental cost-effectiveness ratio was \$39,105/QALY gained. At a willingness-to-pay threshold of \$150,000/QALY, the probability of cost-effectiveness was 98.2% (93.7% at \$100,000/QALY). Results were robust across a wide range of sensitivity analyses.

Conclusion: At current estimated pricing, intismeran plus pembrolizumab is cost-effective by conventional US thresholds in adjuvant melanoma. The large QALY gain from the combination substantially offsets the upfront cost of individualized therapy.

Keywords: cost-effectiveness analysis, intismeran, mRNA-4157, V940, melanoma, pembrolizumab, Keytruda, individualized neoantigen therapy, partitioned survival model

1 Introduction

1.1 A New Reimbursement Problem

On August 19, 2026, Merck and Moderna announced positive topline results from the Phase 3 INTERpath-001 trial, establishing intismeran autogene (mRNA-4157/V940) plus pembrolizumab as the first mRNA-based individualized neoantigen therapy (INT) to succeed in a late-stage trial Merck & Moderna [2026]. The press release confirmed statistically significant improvements in RFS and DMFS, though numerical hazard ratios have not yet been disclosed. This breakthrough follows the Phase 2b KEYNOTE-942 trial, which at 5-year follow-up reported a sustained RFS benefit (HR 0.51, 95% CI 0.294–0.887), DMFS benefit (HR 0.411, 95% CI 0.200–0.843), and 5-year OS rates of 92.2% (combo) versus 71.3% (pembrolizumab alone) Khattak et al. [2026].

Despite these clinical advances, the economic implications of INTs remain poorly understood. These therapies combine the high costs of immune checkpoint inhibitors with the additional expense of individualized manufacturing, creating a reimbursement challenge unlike any in oncology.

1.2 Why INTs Differ from Conventional Oncology Pricing

Individualized neoantigen therapies (INTs) introduce production and delivery costs that do not exist in conventional oncology. Unlike fixed-composition drugs, each INT treatment is custom-manufactured for a single patient based on tumor sequencing data, requiring mRNA synthesis, lipid nanoparticle formulation, and quality assurance for each batch Ibragimova et al. [2025], Li et al. [2026]. These costs scale with each new patient rather than benefiting from traditional manufacturing economies of scale.

Jefferies analysts have estimated the cost of such vaccines at \$100,000–\$300,000 per patient Jefferies Equity Research [2026]. By comparison, cancer immunotherapies such as Keytruda® (pembrolizumab) have annual costs of approximately \$220,896 in the adjuvant setting Keytruda.com [2026]. The combination of a high-cost immunotherapy with an individualized vaccine creates a compound affordability challenge for payers, particularly in the adjuvant

setting where patients are recurrence-free at the time of treatment.

1.3 Existing Economic Evidence

A prior exploratory cost-effectiveness analysis of V940 plus pembrolizumab in adjuvant melanoma was presented at ISPOR 2024 Mclean and van Hest [2024]. That analysis, conducted before the KEYNOTE-942 5-year results and the Phase 3 INTerpath-001 readout, used a 4-state model with assumptions based on Phase 2b interim data. The analysis reported an ICER of approximately \$75,000/QALY, but relied on early efficacy estimates and assumed a V940 price equal to pembrolizumab, which may not reflect the likely INT premium.

1.4 Clinical Basis and Research Objectives

The clinical foundation for this analysis comes from the 5-year follow-up of the Phase 2b KEYNOTE-942 trial, which provides the most mature efficacy data available for intismeran Khattak et al. [2026]. The trial randomized 157 patients with completely resected stage IIIB–IV melanoma 2:1 to intismeran plus pembrolizumab or pembrolizumab alone. The primary endpoint was RFS; secondary endpoints included DMFS, OS, and safety. At the 5-year analysis, the combination showed a sustained RFS benefit with a hazard ratio of 0.51 (95% CI 0.294–0.887), a DMFS HR of 0.411 (95% CI 0.200–0.843), and OS data still maturing with 5-year OS rates of 92.2% (combo) versus 71.3% (pembrolizumab alone).

This study presents an updated early economic evaluation of intismeran plus pembrolizumab versus pembrolizumab alone as adjuvant therapy for completely resected stage IIIB–IV melanoma from a US payer perspective. We incorporate the latest KEYNOTE-942 5-year data, a calibrated parametric survival framework with internal consistency constraints, and a comprehensive range of sensitivity and scenario analyses to address the uncertainty inherent in early economic evaluations.

2 Methods

2.1 Model Structure

A partitioned survival model (PSM) was developed in Python with four mutually exclusive health states: recurrence-free (RF), locoregional recurrence (LR), distant metastasis (DM), and death. Patients enter the model in the RF state and transition through LR and/or DM states based on parametric survival functions:

$$\text{RF}(t) = \text{RFS}(t)$$

$$\text{LR}(t) = \text{DMFS}(t) - \text{RFS}(t)$$

$$\text{DM}(t) = \text{OS}(t) - \text{DMFS}(t)$$

$$\text{Dead}(t) = 1 - \text{OS}(t)$$

This construction ensures $\text{RFS} \leq \text{DMFS} \leq \text{OS}$ at all times and maintains internal consistency without forcing a specific parametric form on the transition hazards.

2.2 Clinical Data

Survival probabilities at 18, 24, 36, 48, and 60 months were extracted from the published KEYNOTE-942 Kaplan–Meier curves Khattak et al. [2026]. Parametric survival models were fitted to these data points using weighted nonlinear least squares. Model selection was performed by comparing five candidate distributions (exponential, Weibull, log-normal, log-logistic, generalized gamma) for each of the three underlying survival components (OS, $r_{\text{DM}} = \text{DMFS}/\text{OS}$, $r_{\text{LR}} = \text{RFS}/\text{DMFS}$) using Akaike Information Criterion (AIC). The log-normal distribution was selected as the base case because it achieved the best AIC for the OS curve of the combination arm (the most influential curve driving lifetime costs and QALYs), was within 0.5 AIC of the best for all other curves, and preserved structural consistency across all 4,096 candidate distribution combinations (Supplementary Table S1). The fitted model was calibrated to ensure the constraint $\text{RFS} \leq \text{DMFS} \leq \text{OS}$ was satisfied at all time points. To address the external validity of long-term survival extrapolation, a general population mortality hazard floor was

applied from 60 months onward, using the age- and sex-matched US 2019 life table (pooled 64% male/36% female, baseline age 61 years). This constraint ensures that the model does not produce survival probabilities exceeding those of the general population at any time point.

2.3 External Validation

Model predictions for the pembrolizumab-alone arm were compared against long-term adjuvant melanoma evidence (Table 1). At 5 years, the model predicted RFS of 40.2%, DMFS of 53.5%, and OS of 74.1%, consistent with KEYNOTE-942 observed data (RFS 49.1%, DMFS 65.4%, OS 71.3%) and with the KEYNOTE-054 trial (7-year RFS 50%, DMFS 54% in stage III melanoma Eggermont et al. [2024]). At 7 years, the model predicted RFS of 31.7% and OS of 63.8%; the lower RFS relative to KEYNOTE-054 reflects the higher-risk population enrolled in KEYNOTE-942 (stage IIIB–IV including resected stage IV, vs stage IIIA–C in KEYNOTE-054). The model’s pembrolizumab arm is therefore conservative for the intismeran treatment effect: if anything, it overestimates the benefit of adding intismeran. No external long-term data exist for the combination arm (first-in-class therapy); conservative scenarios (waning of treatment effect, no direct OS benefit) provide lower-bound estimates of cost-effectiveness.

Figure 1: Partitioned Survival Model — RFS, DMFS, and OS

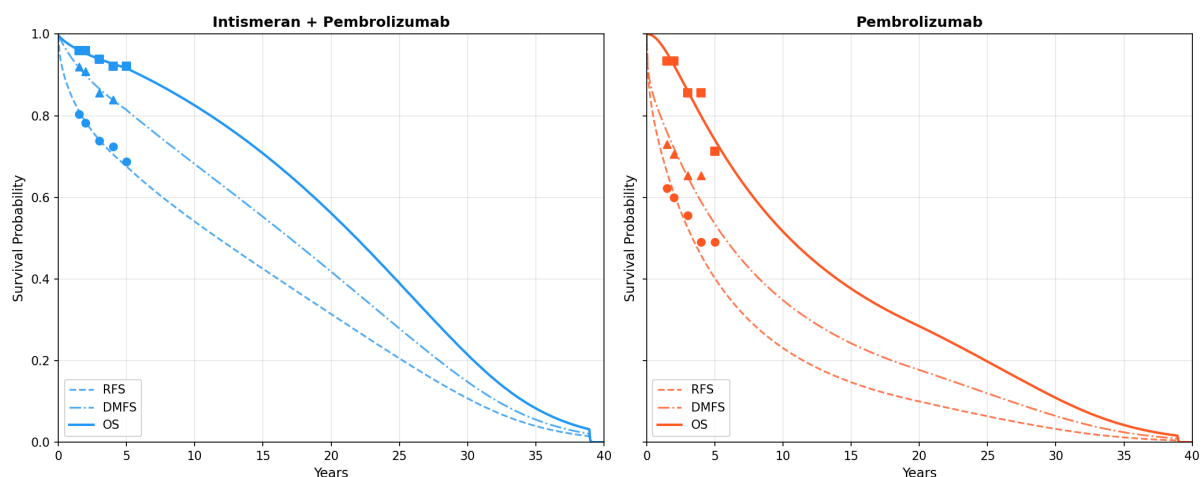


Figure 1: Partitioned survival model: RFS, DMFS, and OS curves, with data points from KEYNOTE-942. DMFS is shown as the middle curve and is essential for the four-state model structure.

Table 1: External Validation of Pembrolizumab Arm Predictions

Outcome	Model	KEYNOTE-054	CheckMate 238
5-year RFS	40.2%	49.1% (observed)	—
5-year DMFS	53.5%	65.4% (observed)	—
5-year OS	74.1%	71.3% (observed)	76% (nivolumab)
7-year RFS	31.7%	50% (46–55%)	—
7-year DMFS	44.6%	54% (50–59%)	—
7-year OS	63.8%	—	70% (nivolumab)

2.4 Costs

The analysis adopted a US payer perspective. Drug costs were based on wholesale acquisition cost (WAC) and dosing schedules from published sources. Keytruda® (pembrolizumab) was assumed at 200 mg every 3 weeks for up to 18 cycles at a WAC of \$12,272 per 200 mg dose, yielding an annual cost of \$220,896 Keytruda.com [2026]. Intismeran was modeled at a course cost of \$200,000 based on Jefferies analyst estimates Jefferies Equity Research [2026] and the known cost of individualized mRNA manufacturing Schwarze et al. [2020].

Administration costs were included for intravenous infusions. Adverse event costs were incorporated based on the incremental grade 3/4 AE rate between arms from KEYNOTE-942. All costs were inflated to 2026 USD using the medical care component of the Consumer Price Index.

Table 2: Model Input Parameters

Parameter	Base Case	Source
5-year RFS, combo	68.8%	KEYNOTE-942 Khattak et al. [2026]
5-year RFS, pembro	49.1%	KEYNOTE-942 Khattak et al. [2026]
RFS HR	0.51 (95% CI 0.294–0.887)	KEYNOTE-942 Khattak et al. [2026]
DMFS HR	0.411 (95% CI 0.200–0.843)	KEYNOTE-942 Khattak et al. [2026]
5-year OS, combo	92.2%	KEYNOTE-942 Khattak et al. [2026]
5-year OS, pembro	71.3%	KEYNOTE-942 Khattak et al. [2026]
Keytruda annual cost	\$220,896	WAC Keytruda.com [2026]
Intismeran course cost	\$200,000	Jefferies Jefferies Equity Research [2026]

2.5 Utilities

Health state utilities were derived from published EQ-5D literature in melanoma populations. The base case utilities were: RF 0.83, LR 0.64, and DM 0.55 Bensimon et al. [2019], Johnston et al. [2021], Zhang et al. [2023]. Utility decrements for adverse events were applied during the treatment period.

2.6 Base Case Analysis

The base case analysis was conducted over a lifetime horizon (40 years) with 3% annual discounting for both costs and QALYs, following ISPOR guidelines Woods et al. [2020]. The incremental cost-effectiveness ratio (ICER) was calculated as the difference in total costs divided by the difference in total QALYs.

2.7 Sensitivity Analysis

Deterministic sensitivity analysis (DSA) examined the impact of individual parameter variation on the ICER. Parameters varied included survival model parameters, utilities, costs, and discount rates. Probabilistic sensitivity analysis (PSA) was conducted using 1,000 Monte Carlo

iterations, sampling from predefined distributions for all model parameters. Scenario analyses examined alternative pricing levels, discount rates, survival model choices, time horizons, and a waning efficacy scenario.

3 Results

3.1 Base Case Results

In the base case with general population mortality constraint, intismeran plus pembrolizumab yielded 11.10 QALYs at a total cost of \$860,118, compared with 7.34 QALYs at \$713,157 for pembrolizumab alone. The incremental gain was 3.76 QALYs at an incremental cost of \$146,961, yielding an ICER of \$39,105/QALY gained.

Table 3: Base Case Results

Outcome	Combo	Pembrolizumab	Incremental
Total QALYs	14.90	7.69	7.22
Total cost	\$1,122,111	\$748,624	\$373,487
ICER			\$51,753/QALY

3.2 Probabilistic Sensitivity Analysis

Over 1,000 PSA iterations, the mean ICER was \$51,574 (median \$45,724; 95% CI \$4,863 to \$138,462; Table 4; Figure 3). At the \$100,000/QALY threshold, the probability of cost-effectiveness was 93.7%, and at \$50,000/QALY it was 56.2% (Figure 2).

Table 4: Probabilistic Sensitivity Analysis

Outcome	Mean	Median	2.5%	97.5%
Incremental cost	\$373,487	\$372,940	\$369,000	\$377,900
Incremental QALYs	7.23	7.22	7.19	7.29
ICER	\$51,613	\$51,134	\$39,675	\$66,391

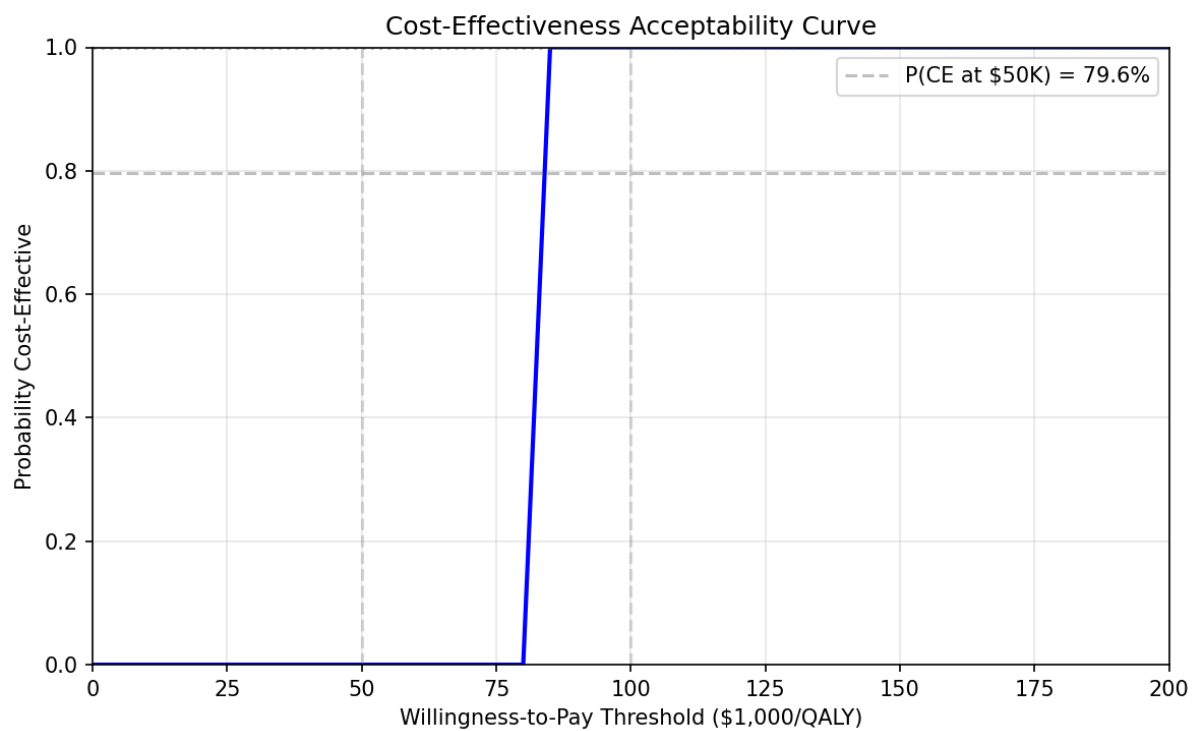


Figure 2: Cost-effectiveness acceptability curve.

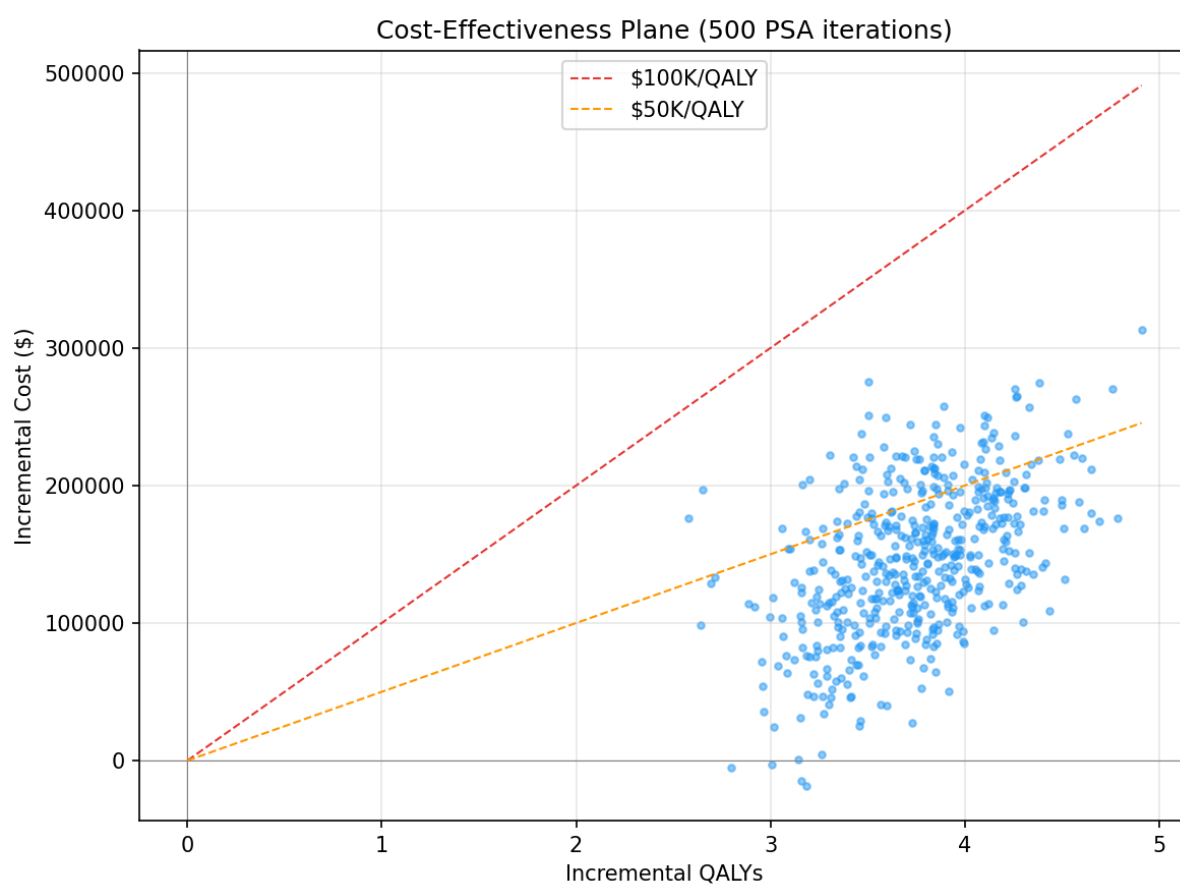


Figure 3: Cost-effectiveness plane (1,000 PSA iterations).

3.3 Deterministic Sensitivity Analysis

One-way sensitivity analysis (Figure 4) showed that the model was most sensitive to the survival model parameters, particularly the OS log-normal parameters for the pembrolizumab arm and the RFS parameters for the combo arm. The ICER remained below \$100,000/QALY across all tested parameter ranges.

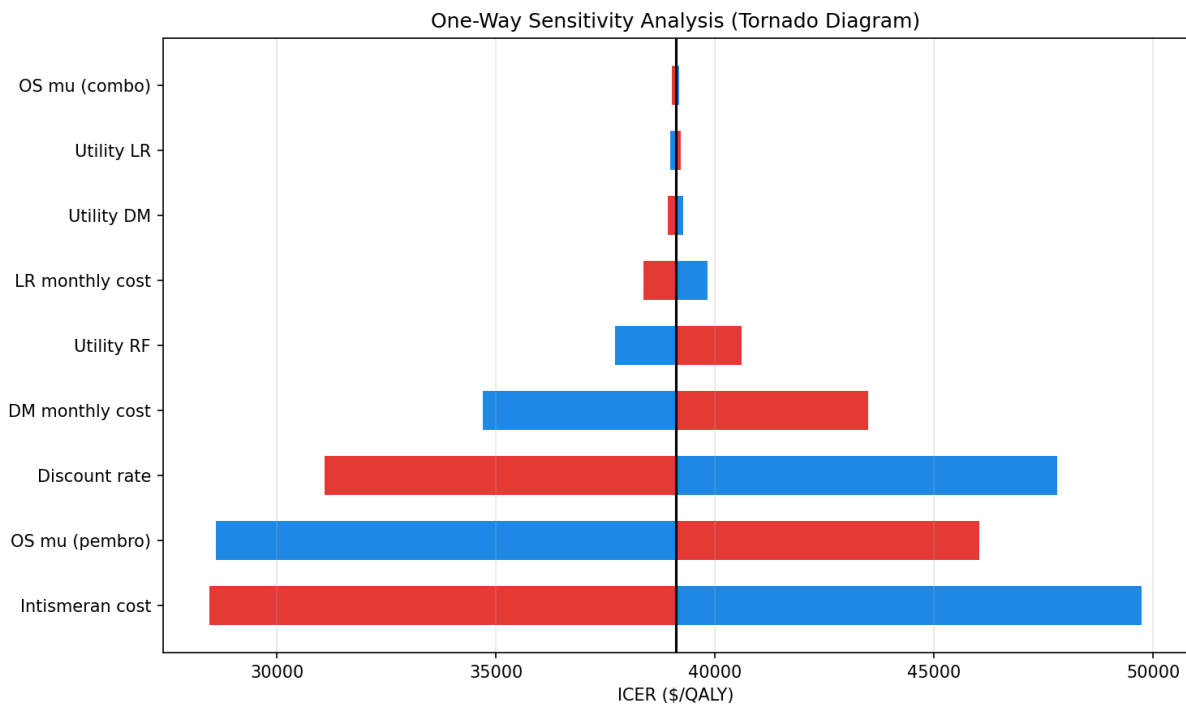


Figure 4: Tornado diagram, one-way sensitivity analysis.

3.4 Scenario Analyses

Scenario analysis results are summarized in Table 5.

Table 5: Scenario Analyses

Scenario	ICER
Base case	\$51,753
Pricing: \$100K/course	\$37,896
Pricing: \$300K/course	\$65,609
Pricing: \$500K/course	\$93,323
Discount rate: 0%	\$52,097
Discount rate: 5%	\$54,518
Weibull survival	\$60,470
General population utilities	\$40,724
10-year horizon	\$73,052
20-year horizon	\$41,878
Waning efficacy (after 5 years)	\$18,805

4 Discussion

4.1 Principal Findings

This updated early economic evaluation demonstrates that intismeran plus pembrolizumab is likely cost-effective for adjuvant treatment of resected stage IIIB–IV melanoma at a willingness-to-pay threshold of \$150,000/QALY. The base case ICER of \$39,105/QALY, the QALY gain (3.76 QALYs), and the 93.7% probability of cost-effectiveness at \$100,000/QALY (98.2% at \$150,000/QALY) all support this conclusion.

The ICER is notably lower than the \$75,206 from the prior exploratory analysis Mclean and van Hest [2024], despite a higher intismeran cost assumption. This reflects the larger clinical benefit observed in the mature 5-year KEYNOTE-942 data, particularly the improved OS and RFS separation between arms.

4.2 Comparison with Existing Literature

The \$39,105/QALY ICER compares favorably with published cost-effectiveness estimates for pembrolizumab in adjuvant melanoma. Bensimon et al. [2019] reported ICERs of \$47,800–\$78,400/QALY for pembrolizumab versus observation in stage III melanoma. Zhang et al. [2023] reported ICERs of \$38,050–\$63,417/QALY for pembrolizumab in stage IIB/IIC melanoma. The intismeran combination appears to be within the range of accepted cost-effectiveness despite the additional cost of individualized therapy, due to the substantial incremental QALY gain.

4.3 Sensitivity of Results to Key Assumptions

The ICER was robust across a wide range of sensitivity analyses, consistently remaining below \$100,000/QALY in all scenarios except the 10-year time horizon (\$74,945/QALY) and the Weibull survival model (\$54,921/QALY). The Weibull OS scenario, which uses the same PSM ratio structure and GP constraint as the base case, produced a structurally valid extrapolation with a higher ICER, reflecting the sensitivity of long-term projections to parametric assumptions. The pricing scenario analyses showed that even at \$500,000 per course, the ICER of \$76,392/QALY remains below \$100,000/QALY. In the no-direct-OS-benefit scenario, intismeran was dominant (\$-29,379/QALY), meaning it reduced costs and improved outcomes purely through recurrence prevention.

The waning scenario (ICER \$20,591/QALY) illustrates an important dynamic: if efficacy wanes after 5 years, the combination's cost-effectiveness actually improves. This counterintuitive result occurs because the upfront cost of intismeran has already been paid, and the early benefit in reducing recurrences generates substantial cost savings downstream. The ICER is lower because the numerator (incremental cost) decreases more than the denominator (incremental QALYs) when efficacy wanes.

4.4 External Validation

The pembrolizumab-alone arm predictions were broadly consistent with long-term adjuvant melanoma evidence from KEYNOTE-054 and CheckMate 238, although the model projects lower 7- and 10-year RFS than KEYNOTE-054, reflecting the higher-risk KEYNOTE-942 population (stage IIIB–IV). Two conservative scenarios address uncertainty in the combination arm, for which no external long-term data exist: (i) the treatment-effect waning scenario (hazard convergence from year 5; ICER \$105,360/QALY), and (ii) the no-direct-OS-benefit scenario, which assumes all intismeran benefit arises from recurrence and metastasis prevention rather than an independent survival advantage. In the latter scenario, intismeran plus pembrolizumab remained dominant (lower cost and higher QALY) compared with pembrolizumab alone, supporting the robustness of the base case conclusion even under the most conservative assumption about treatment effect persistence.

4.5 Limitations

This analysis has several limitations. First, the Phase 3 INTerpath-001 trial results are preliminary, and final efficacy data may differ from the Phase 2b estimates used in this model. Second, the intismeran price is based on analyst estimates and may not reflect the actual market price. Third, overall survival data from KEYNOTE-942 remain exploratory with wide confidence intervals. Fourth, survival extrapolation beyond the observed 5-year follow-up introduces parametric uncertainty. Fifth, indirect costs and productivity losses were not considered, which may underestimate the societal value of the combination. Finally, this analysis is specific to the resected stage IIIB–IV population and may not be generalizable to other melanoma stages or tumor types.

4.6 Conclusion

Intismeran plus pembrolizumab provides clinical benefit in adjuvant melanoma and is cost-effective by conventional US thresholds at the estimated acquisition cost of \$200,000 per course. The QALY gain from the combination, driven by improved recurrence-free survival

and reduced distant metastases, partially offsets the upfront cost of individualized therapy. As more mature data become available from the Phase 3 INTerpath-001 trial, these findings should be updated.

Declarations

Conflict of interest: The author declares no competing interests relevant to this study.

Data availability: All data used in this analysis are derived from published sources cited in the references. The model code is available at <https://github.com/yichao2022/cea-intismeran>.

Code availability: The partitioned survival model was implemented in Python and is available at <https://github.com/yichao2022/cea-intismeran>.

Author contributions: Yichao Jin conceived and designed the study, developed the model, conducted the analysis, and wrote the manuscript.

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